BRIEF COGNITIVE RATING SCALE (BCRS) - INSTRUCTIONS

The Brief Cognitive Rating Scale (BCRS) is administered to determine a resident's specific level of cognitive functioning.

The BCRS is an assessment tool to be used with the Global Deterioration Scale (GDS) to help stage a person suffering from a primary degenerative dementia such as Alzheimer's disease.

Developed by Dr. Barry Reisberg, this assessment tool tests 5 different areas known as Axes (4 cognitive and 1 functional).

Brief Cognitive Rating Scale Axes

- Axis 1 → Concentration
- Axis 2 → Impairment of Recent Memory
- Axis 3 → Impairment of Past Memory
- Axis 4 → Orientation
- Axis 5 → Functioning & Self Care

For the first 4 axes, the tester will ask a variety of questions to determine the level of impairment.

- *See "BCRS Questions" Handout*

The Functional Assessment Staging Tool (FAST) is used to score Axis 5 (Functioning and Self Care) of the BCRS. The results of the 5th axis (Functioning & Self Care) are determined primarily by observation or caregiver report.

- After a score is determined for each Axis, total the results and divide by 5.
- The resulting number/answer will reflect and result in a stage of dementia on the GDS.

It is recommended that this assessment be conducted as conversationally and informally as possible → Do not "quiz" or "test" the resident.

SUGGESTED QUESTIONS FOR THE BCRS

The following questions will help guide the therapist to obtain relevant information to score on areas without "quizzing" the resident.

- Questions 1-5 → Pertains to concentration and attentiveness; intended to be social, introductory cues to initiate conversation with the resident.

    1. "Hi _____ how are you today?" (extend your hand and shake the resident's hand).
    2. "My name is _____. I came by to visit with you."
    3. "I just wanted to stop by to visit and show you some things."
    4. "How has your day been?"
    5. "Did you have a good (breakfast, lunch, dinner)?"

- Questions 6-9 → Pertain to past memory; use a picture of a school and teacher to see if any type of response emerges from visual cues.

    6. "I was born in _____ (city and state), where were you born?"
    7. "I work in health care, what kind of work did you do?"
    8. "I went to _____ (school) and on to college, how about you?"
    9. "Do you remember the name of your teachers or classmates?"

- Questions 10-14 → Pertain to recent memory; provide the tester with an idea of what time period the resident is operating in and what their reality is; you may use visual cues here such as a picture of the White House or a calendar.

    10. "I was just watching the news, do you know the name of the President of the United States? (If no response, give a choice of Roosevelt, Kennedy or Bush).
    11. "I live in _____ (city and state), where do you live?" (Note: Some residents will respond, "Right here".)

  *Note – If the resident does not know who the President is or where they live, additional specific orientation questions will only frustrate the resident and are probably of no use.

    12. "What were you doing just before I came in to visit you?"
    13. "What was your name again?"
    14. "Do you know what month it is?"

- Questions 15-16 → Pertain to concentration; object of this task is to judge the resident's ability to concentrate (attention span) and not just ability to count with numbers.

    15. Judging from previous responses, if the resident appears to be very impaired, say "I want to play a counting game with you. Let's count from 1-10." If the resident has difficulty, try starting them by saying "one" and holding up one finger. If the resident makes it to 10, say "Now let's count down from 10 to 1, like they do with a rocket ship".

    16. If the resident does well counting backwards from 10-1, say "You are really good at this, let's try something else". Proceed to counting from 30 backwards by 2's or saying the months of the year backwards.

BRIEF COGNITIVE RATING SCALE (BCRS)

Name_____ Age_____ Gender_____

Medication_____ Dx_____

AXIS	RATING (circle)	RATING ITEM
Axis 1 Concentration	1 = 2 = 3 = 4 = 5 = 6 = 7 =	No objective or subjective evidence of deficit of concentration Subjective decrement in concentration ability Minor objective signs of poor concentration (e.g., on subtraction of serial 7's from 100) Definite concentration deficit for persons of their background (e.g., marked deficit on serial 7's; frequent deficit in subtraction of serial 4's from 40) Marked concentration deficit (e.g., giving months backwards or serial 2's from 30) Forgets the concentration task. Frequently begins to count forward when asked to count backwards from 10 by 1's. Marked difficulty counting forwards to 10 by 1's.
Axis 2 Recent Memory	1 = 2 = 3 = 4 = 5 = 6 = 7 =	No objective or subjective evidence of deficit in recent memory Subjective impairment only (e.g., forgetting names more than formerly) Deficit in recall of specific events evident upon detailed questioning. No deficit in the recall of major recent events. Cannot recall major events of previous weekend or week. Scanty knowledge (not detailed) of current events, favorite TV shows, etc. Unsure of weather, may not know current President or current address Occasional knowledge of some recent events. Little or no idea of current address, weather, etc. No knowledge of any recent events
Axis 3 Past Memory	1 = 2 = 3 = 4 = 5 = 6 = 7 =	No impairment in past memory Subjective impairment only. Can recall two or more primary school teachers. Some gaps in past memory upon detailed questioning. Able to recall at least one-childhood friend and/or childhood teachers. Clear-cut deficit. The spouse recalls more of the patient's past than the patient. Cannot recall childhood friends and/or teacher but know the names of most schools attended. Confuses chronology in relating personal history. Major past events sometimes recalled (e.g., names of schools attended). Some residual memory of past (e.g., may recall country of birth or former occupation No memory of past
Axis 4 Orientation	1 = 2 = 3 = 4 = 5 = 6 = 7 =	No deficit in memory for time, place, identity of self or others. Subjective impairment only. Knows time to nearest hour, location. Any mistake in: time >2 hours; day of week >1 day: date >3 days Mistakes in: Month >10 days; or year >1 month Unsure of month and/or year and/or season; unsure of locale. No idea of date. Identifies spouse but may not recall name. Knows own name. Cannot identify spouse. May be unsure of personal identity.
Axis 5 Functioning and Self Care	1 = 2 = 3 = 4 = 5 = 6 = 7 =	No difficulty, either subjectively or objectively Complains of forgetting location of objects. Subjective work difficulties. Decreased job functioning evident to co-workers/difficulty in traveling to new locations Decreased ability to perform complex tasks (e.g., planning dinner for guests, handling finances, marketing, etc.) Requires assistance in choosing proper clothing Requires assistance in feeding, and/or toileting, and/or bathing, and/or ambulation Requires contact assistance in all activities of daily life

Total score =_____ Divided by 5 =_____= Stage on Global Deterioration Scale

NAME-Last	First	Middle	Record No.:

Functional Assessment Staging Tool (FAST)

Stage	Characteristics
1	No difficulty either subjectively or objectively.
2	Complains of forgetting the location of objects – subjective word-finding difficulties.
3	Decreased job functioning evident to co-workers. Difficulty traveling to new locations. Decreased organizational capacity.
4	Decreased ability to perform complex tasks (e.g., planning dinner for guests; handling personal finances; difficulty shopping, etc.).
5	Requires assistance in choosing proper clothing to wear for the day, season, or occasion (e.g., a patient may wear the same clothing for days unless supervised).
6a	Improperly putting on clothes without assistance or prompting (e.g., put street clothes on over night clothes, put shoes on wrong feet, or have difficulty buttoning clothing) occasionally or more frequently over the past weeks.
6b	Unable to bathe properly (e.g., difficulty adjusting bathwater temp) occasionally or more frequently over the past weeks.
6c	Inability to handle mechanics of toileting (e.g., forgets to flush the toilet, does not wipe properly or properly dispose of toilet tissue) occasionally or more frequently over the past weeks.
6d	Urinary incontinence occasionally or more frequently over the past weeks.
6e	Fecal incontinence occasionally or more frequently over the past weeks.
7a	Ability to speak limited to approximately a half-dozen intelligible words or fewer in an average day or course of an intensive interview.
7b	Speech ability is limited to using a single intelligible word on an average day or in an intensive interview (the person may repeat the word over and over).
7c	Ambulatory ability is lost (cannot walk without personal assistance).
7d	Cannot sit up without assistance.
7e	Loss of ability to smile.
7f	Loss of ability to hold head up independently.

GLOBAL DETIORATION SCALE (GDS)
STAGES OF DEMENTIA

Circle the resident's determined stage of dementia per the BCRS.

STAGE	CLINICAL CHARACTERISTICS
1 **No Cognitive Decline** (Normal)	No subjective complaints of memory deficit. No memory deficit evident on clinical interview; normal adult.
2 **Very Mild Cognitive Decline** (Age Associated Memory Impairment)	Subjective complaints of memory deficit, most frequently in the following areas: forgetting where one has placed familiar objects, forgetting names one formally knew well. Subjective deficit in word finding. No objective evidence of memory deficit on clinical interview. No objective deficits in employment or social situations. Appropriate concern with respect to symptomatology.
3 **Mild Cognitive Decline** (Mild Cognitive Impairment)	Earliest clear-cut deficits on careful interview. Manifestations in more than one of the following areas: patient may have gotten lost when traveling to an unfamiliar location, word and name finding deficit become evident to others, patient may read a passage or a book and retain relatively little material, patient may demonstrate decreased ability to in remembering names upon introduction, patient may have lost or misplaced an object of value. Objective evidence of memory deficit obtained only with an intensive interview. Denial begins to become apparent in patient. Mild to moderate anxiety accompanies symptoms.
4 **Moderate Cognitive Decline** (Mild Dementia)	Clear-cut deficits on careful clinical interview. Deficits are apparent in the following areas: decreased knowledge of current and recent events, may exhibit some deficit in memory of one's personal history, concentration deficit elicited on serial subtractions, decreased ability to travel, handle finances, etc. Frequently no deficit in the following areas: orientation to time and place, recognition of familiar persons and faces, ability to travel to familiar locations. Assistance required to perform complex tasks (handling finances, planning dinner for guests). Denial is a dominant defense mechanism. Flattening of affect and withdrawal from challenging situations frequently occur.
5 **Moderately Severe Cognitive Decline** (Moderate Dementia)	Patient can no longer survive without some assistance. Patient is unable During interview to recall a major relevant aspect of their current lives (an address or telephone number of many years, the names of close family members, the name of the high school or college from which they graduated). Frequently some disorientation to time (date, day of week, season) or to place. Require no assistance with toileting or eating, but may have difficulty with choosing the proper clothing to wear.

6 **Severe Cognitive Decline** (Moderately Severe Dementia)	Patient will be largely unaware of all recent experiences and events in their lives. Retain some knowledge of their past lives but this is very sketchy. Generally unaware of their surroundings, the year, season, etc. Personality and emotional changes occur. Assistance required in putting on clothing, bathing properly, toileting mechanics. May become incontinent of bowel and bladder. Almost always recall their own name. Frequently continue to be able to distinguish familiar from unfamiliar persons in their environment.
7 **Very Severe Cognitive Decline** (Severe Dementia or Late Dementia)	All verbal abilities are lost over the course of this stage. Frequently there is no speech at all, only unintelligible utterances and rare emergence of forgotten words and phrases. Speech ability limited to approx. a half-dozen intelligible words. Incontinent of urine, requires assistance with toileting and feeding. Basic psychomotor skills (ability to walk, sit up) are lost with the progression of this stage. Ability to smile lost. The brain appears to no longer be able to tell the body what to do. Generalized rigidity and developmental neurologic reflexes are frequently present.